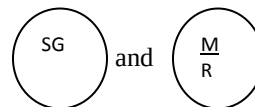


DIMAX MR TABLET 30MG

Gliclazide 30mg (modified release tablet)

Description

Dimax MR tablets 30mg are white, round tablets and the surface of the tablet are



Composition

Each tablet contains 30mg of Gliclazide.

Action

SULFONAMIDES, UREA DERIVATIVES

ATC code: A10BB09

Gliclazide is a hypoglycaemic sulphonylurea oral antidiabetic active substance differing from other related compounds by an N-containing heterocyclic ring with an endocyclic bond.

Gliclazide reduces blood glucose levels by stimulating insulin secretion from the β -cells of the islets of Langerhans. Increase in postprandial insulin and C-peptide secretion persists after two years of treatment.

In addition to these metabolic properties, gliclazide has haemovascular properties.

Summary of pharmacodynamics and pharmacokinetics

Gliclazide is readily absorbed from the gastrointestinal tract and extensively bound to plasma proteins. The half-life is about 10 to 12 hours. Gliclazide is extensively metabolized in the liver to metabolites that have no significant hypoglycemic activity. Metabolites and a small amount of unchanged drug are excreted in the urine.

Indications

Gliclazide is indicated for the treatment of non insulin-dependent diabetes (type 2) in adults when dietary measures, physical exercise and weight loss alone are not sufficient to control blood glucose.

Adverse reactions/side effects

Gastrointestinal disturbances such as nausea, vomiting, heartburn, anorexia, diarrhea and metallic taste may occur with Gliclazide and are usually mild and dose dependent; increased appetite and weight gain may occur. Skin rashes and pruritus and photosensitivity have been reported. Rashes are usually hypersensitivity reactions and may progress to more serious disorders. Other severe effects may be manifestations of hypersensitivity reaction. They include altered liver enzyme values, hepatitis and cholestatic jaundice, leucopenia, thrombocytopenia, aplastic anemia, agranulocytosis, haemolytic anemia, erythema multiforme or the Stevens-Johnson syndrome, exfoliative dermatitis, and erythema nodosum.

Mild hypoglycemia may occur; severe hypoglycemia is usually caused by overdosage and is relatively uncommon.

Precautions/warnings

Hypoglycemia: As with other sulphonylureas, gliclazide is susceptible to cause episodes of hypoglycemia (decrease of blood sugar levels). In the case of sweating, intense hunger, trembling, pallor, visual disturbances, feeling of malaise, abnormal behavior, immediately eat sugar or something containing sugar and inform the doctor.

Hypoglycemia is favored by a strict or poorly balanced diet, by prolonged or strenuous exercise, by the intake of alcohol or during use of other hypoglycemic drugs (see Interactions).

In consequence, treatment with a hypoglycemic sulphonylurea requires: Regular Diet: It is important to eat regular meals, including breakfast, never to miss a meal, due to an increase in the risk of the onset of hypoglycemia caused by an inadequate diet or a sugar imbalance.

Precise Regularity in the Administration of Treatment: It is important to take treatment regularly, once a day, preferably at breakfast (see Dosage & Administration).

Every administration of Dimax MR must be accompanied by a meal.

Contraindications

Dimax tablet MR tablet 30mg contraindicated to patients with hypersensitivity to gliclazide or to other medicines of the same group (sulfonylureas); sulfonamides. Diabetes requiring treatment with insulin or insulin-dependent diabetes (type 1); ketoacidosis, diabetic precoma, severe liver or kidney disease, treatment with miconazole. Unless the doctor advises the patient on the contrary, it is generally inadvisable to combine gliclazide with phenylbutazone or danazol (see also Interactions).

Dosage and directions for use

Dimax MR tablet is administered by oral.

For adult use only.

The daily dose may vary from 1 to 4 tablets per day, *i.e.* from 30 to 120 mg taken orally in a single intake at breakfast time.

It is recommended that the tablet(s) be swallowed whole.

If a dose is forgotten, there must be no increase in the dose taken the next day.

As with any hypoglycaemic agent, the dose should be adjusted according to the individual patient's metabolic response (blood glucose, HbA1c)

Initial dose

The recommended starting dose is 30 mg daily.

If blood glucose is effectively controlled, this dose may be used for maintenance treatment.

If blood glucose is not adequately controlled, the dose may be increased to 60, 90 or 120 mg daily, in successive steps. The interval between each dose increment should be at least 1 month except in patients whose blood glucose has not reduced after two weeks of treatment. In such cases, the dose may be increased at the end of the second week of treatment.

The maximum recommended daily dose is 120 mg.

Switching from immediate release 80 mg tablets to Dimax 30 mg modified release tablets:

1 tablet of immediate release 80 mg is comparable to 1 tablet of Dimax 30 mg MR Tablets (Gliclazide Modified Release 30mg tablets). Consequently the switch can be performed provided a careful blood monitoring is done.

Switching from another oral antidiabetic agent to Dimax 30 mg MR Tablets:

Dimax 30 mg MR Tablets can be used to replace other oral antidiabetic agents.

The dosage and the half-life of the previous antidiabetic agent should be taken into account when switching to Dimax 30 mg MR Tablets.

A transitional period is not generally necessary. A starting dose of 30 mg should be used and this should be adjusted to suit the patient's blood glucose response, as described above.

When switching from a hypoglycaemic sulphonylurea with a prolonged half-life, a treatment free period of a few days may be necessary to avoid an additive effect of the two products, which might cause hypoglycaemia. The procedure described for initiating treatment should also be used when switching to treatment with Dimax 30 mg MR Tablets, *i.e.* a starting dose of 30 mg/day, followed by a stepwise increase in dose, depending on the metabolic response.

Combination treatment with other antidiabetic agents:

Dimax 30 mg MR Tablets can be given in combination with biguanides, alpha glucosidase inhibitors or insulin.

In patients not adequately controlled with Dimax 30 mg MR Tablets, concomitant insulin therapy can be initiated under close medical supervision.

In the elderly (over 65), Dimax 30 mg MR Tablets should be prescribed using the same dosing regimen recommended for patients under 65 years of age.

Impaired Renal function in patients with mild to moderate renal insufficiency the same dosing regimen can be used as in patients with normal renal function with careful patient monitoring. These data have been confirmed in clinical trials.

In patients at risk of hypoglycaemia:

- undernourished or malnourished,
- severe or poorly compensated endocrine disorders (hypopituitarism, hypothyroidism, adrenocorticotrophic insufficiency),
- withdrawal of prolonged and/or high dose corticosteroid therapy,

- severe vascular disease (severe coronary heart disease, severe carotid impairment, diffuse vascular disease);
It is recommended that the minimum daily starting dose of 30 mg is used.

Children

There are no data and clinical studies available in children.

Use in pregnancy and lactation

If the patient is pregnant, diabetes should be treated with insulin. Inform the doctor if the patient wishes to become pregnant. If the patient is pregnant, inform the doctor so that he may prescribe a more suitable treatment. If the patient discovers that she is pregnant while taking Dimax MR, discontinue treatment.

It is not known whether gliclazide or its metabolites are excreted in breast milk. Given the risk of neonatal hypoglycaemia, the product is contra-indicated in breast-feeding mothers.

Drug interactions

Gliclazide and other sulfonylureas hypoglycemic effect will be diminished and possibly requiring an increased dose, has seen or might expected with adrenaline, aminoglutethimide, corticosteroids, chlorpromazine, diazoxide, rifamycins, oral contraceptives, thiazide diuretics and thyroid hormones. An increased with hypoglycemic effects has occurred or might be expected with ACE inhibitors, alcohol, allopurinol, some analgesics (notably azapropazone, phenylbutazone and the salicylates), azole antifungals (fluconazole, ketoconazole and miconazole), chloramphenicol, cimetidine, clofibrate and related compounds, coumarins anticoagulants, flouroquinolones, heparin, MAOI's, octreotide (although this may also produce hyperglycemia), ranitidine, sulfinpyrazone, sulfonamides (including co-trimoxazole), tetracyclines and tricyclic antidepressants.

Beta blockers have been reported both to increased hypoglycemia and to mask the typical sympathetic warning signs.

Overdosage, symptoms and treatment

An overdose of sulphonylureas may cause hypoglycaemia.

Moderate symptoms of hypoglycaemia, without any loss of consciousness or neurological signs, must be corrected by carbohydrate intake, dose adjustment and/or change of diet. Strict monitoring should be continued until the doctor is sure that the patient is out of danger.

Severe hypoglycaemic reactions, with coma, convulsions or other neurological disorders are possible and must be treated as a medical emergency, requiring immediate hospitalisation.

If hypoglycaemic coma is diagnosed or suspected, the patient should be given a rapid I.V. injection of 50 ml of concentrated glucose solution (20 to 30 %). This should be followed by continuous infusion of a more dilute glucose solution (10 %) at a rate that will maintain blood glucose levels above 1 g/L. Patients should be monitored closely and, depending on the patient's condition after this time, the doctor will decide if further monitoring is necessary.

Dialysis is of no benefit to patients due to the strong binding of gliclazide to proteins.

Presentation

This tablet is available in 100's

10 tablets are packed into one blister

Storage conditions

Protect from light and store below 30°C. Keep out of reach of children.

Shelf-life

The tablets can be used within 24 months from the date of manufacture if kept as recommended.

Registration number

DIMAX MR TABLET 30MG – MAL12045002AZ

Manufactured by

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